جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : تهاني جمعه كامل سيد
المحافظة : الجيزه **الحي / المدينة :** الصف **العنوان :** الصف
رقم البطاقة : 27311212100967 **رقم التليفون :** 01025501722 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 21-11-1973 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: تهاني جمعه كامل سيد
National ID: 27311212100967

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية

بطاقة تحقيق الشخصية

تهانى

جمعه كامل سيد خطاب

قرية كفر طرخان

مركز الصف - الجيزة

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ربة منزل

انثى مسلمة

١ البطاقة سارية حتى ٢٠٣١/٠٥/٢٥



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	: الأستاذه / تهاني جمعه كامل سيد	Request No.	: 13006688
Patient No.	: 27311212100967	Request Date	: 08/08/2023
Path No.	: 2559/23	Verification Date	: 28/08/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 49 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right UOQ breast mass, BIRAD 5.
Nature of Specimen: Tru-cut biopsy, collected at 7/8/2023, 10:46 am.

Gross Examination:
- Multiple grayish white tissue cores up to 1 cm long , totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma .
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III
* Ductal carcinoma in situ: No .
* Perineural invasion: No .
* Angioinvasion: No .

* IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in the tumor cells (Allred score 0/8).
- PR: No nuclear reaction in the tumor cells (Allred score 0/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- KI67: 80% nuclear reaction of tumor cells.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma , Grade II.
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).

Verified by :
Dr.Marwa mahmoud

Room No. 40 - First Floor-Building No. 383- Nile Corniche - Sitar El Qudessia- Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: تهاني جمعه كامل سيد	
MR. : 5539	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presented with a left breast lesion noted in January 2026; imaging identified a left breast lipoma with both lesions classified as BIRADS 2. The attending recommended full laboratory work-up and a CT of the chest, abdomen, and pelvis for comprehensive assessment.

The patient subsequently received neoadjuvant chemotherapy consisting of 4 cycles of AC followed by 12 weeks of taxol, achieving a complete radiological response. After neoadjuvant therapy, the patient underwent breast-conserving surgery with sentinel lymph node biopsy, resulting in ypT0 ypN0 pathology. Post-operative radiotherapy was administered. At follow-up, the patient reports vague diffuse pains and headache; CT CAP is unremarkable and laboratory studies are within normal limits. The plan is follow-up in three months with repeat labs and a bone scan. The patient has completed four cycles of AC and twelve weeks of taxol.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : ندى حسن صلاح كامل

يعتمد ,

Patient Name	:الأستاذه / تهانى جمعه كامل سيد	Request No.	: 13006688
Patient No.	:27311212100967	Request Date	: 08/08/2023
Path No.	:2559/23	Verification Date	: 28/08/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	:49 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

- Her2: Negative (score 0).
- Ki67: Positive in 80% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Rana fathy
Dr. Menan Hossam

Verified by :
Dr.Marwa mahmoud

Room No. 68 - First Floor-Building No. 247- 90th Corniche - Nile El Qadeem- Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نهالى جمعه نامل سيد	Request No.	: 13008587
Patient No.	: 27311212100967	Request Date	: 02/04/2024
Path No.	: 1220/24	Verification Date	: 16/04/2024
Sex	: Female	Financial Category	: اورام دار السلام - ه اده
Age	: 50 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, post NACT.
Nature Specimen: Right WLE and SLNs.

Gross Examination:
* Right SLN: A fibro fatty tissue piece measured 3x3x3 cm entangling 4 nodules up to 2x1 cm.
* Right WLE specimen measured 10x9x3 cm, with covering skin ellipse measured 5.5x2.5 cm
* Tumor extent: A wired and clipped firm grayish white tumor bed site.
- Size: 1.5x1x1 cm
* Distance from margins: 1 m from (superior and inferior margins)

Microscopic Examination:
* Right breast, WLE and SLNs biopsy:
* Histological Type: Total tumor regression.
* Ductal carcinoma in situ: No.
* Lymph node involvement: Free SLNs (0/4).
* Angioinvasion: No.
* Perineural invasion: No.
* Resection Margins: Free margins.
* Histologic response to therapy: Complete pathological response.
*Additional pathological finding: Effect of therapy in the foarm of fibrosis, hyalinosis and histiocytic reaction.
* Frozen section results:
Free margins.
Free SLNs (0/4).

Diagnosis:
Right breast, WLE and SLNs Biopsy:

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

ent Name	:الأستاذه / تهانى جمعه نامل سيد	Requ... No.	: 3008587
ent No.	:27311212100967	Request Date	: 02/04/2...
No.	:1220/24	Verification Date	: 16/04/2024
	:Female	Financial Category	: مبادره - اورام دار السلا
	:50 Y	Department	:Out Patient
erreing Doc :		Registered by	:.marwa mohamed youss

Pathology Report

al tumor regression.
e margins.
ie SLNS (0/4).
mplete pathologʻical response.
ıge: ypT0, ypN0.

Signature
Prof .Dr. Gamal Abdelatty.
Dr. Marwa Mahmoud.

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

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Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Tahany Gomaa Kamel Sayed	Date : 22/04/2025
Exam : C.T CHEST	Age : 51 Year(s)
Medical ID: 3266/2023	Pat. ID: 41491

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. ***Chest: **Status post right breast CBS . As compared to previous study at 16-09-2024***

Findings:

- **Regressive course regarding size** of previously noted Right breast thick walled deeply seated seroma is seen measuring 1.9x3 cm as compared to 2.8 X 5.4 cm in prior study.
- Currently appreciated middle lung lobe subpleural reticulations likely post radiation sequel
- Stationary course regarding size right basal lateral segment nodule still measuring 3 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Bilateral small fibroelastoma dorsi are still noted.
- Upper abdominal cuts are unremarkable.

2. ***Abdomen and pelvis:***

- The liver is average in size with homogenous CT texture. No definite hypodense focal lesions seen, no contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.

CT Scan
[[DOCTYPE:CT_SCAN]]

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مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Tahany Gomaa Kamel Sayed	Date : 16/09/2024
Exam : C.T CHEST	Age : 51 Year(s)
Medical ID: 3266/2023	Pat. ID: 41491

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Findings:

- Right breast thick walled deeply seated seroma is seen measuring 2.8 X 5.4 cm.
- Stable right basal lateral segment nodule measuring 3 mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Upper abdominal cuts are unremarkable.
- D1 upper vertebral endplate subchondral lucencies.

OPINION:

- ***No definite CT evidence of pulmonary metastatic deposits.***

Dr.Ahmed Elgazar

Dr. Mohamed Mohsen, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

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مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Tahany Gomaa Kamel Sayed	
Exam : MRI BRAIN WITH CONTRAST	Age : 51 Year(s)
Medical ID: 6632/2023	41491

MRI STUDY OF THE BRAIN

Technique of Examination:

- *Axial T1, T2 & FLAIR WIs*
- *Sagittal T1WIs*
- *Post contrast series*

MR Findings:

- No definite enhancing intra cranial masses.
- No cerebral parenchymal areas of abnormal signal, yet few frontoparietal tiny foci of abnormal signal showing high T2/FLAIR signal with no surrounding mass effect.
- Normal grey / white matter differentiation.
- Normal size and configuration of the ventricular system.
- No shift of midline structures.
- No areas of fresh blood or blood degradation product signal.
- Normal appearance of the brain stem and cerebellum.
- No extra axial collections.
- *NB. Mild right maxillary sinusitis noted.*

Opinion:

- **No definite enhancing intra cranial masses.**
- **Few frontoparietal tiny ischemic foci (likely non-recent) for clinical correlation.**

Dr. Sherif Sameh, M.D.

M.R.I
[[DOCTYPE:MRI]]

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مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Tahany Gomaa Kamel Sayed	Date : 01/09/2024
Exam : MAMMOGRAPHY BOTH	Age : 50 Year(s)
Medical ID: 3266/2023	Pat. ID: 41491

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had CBS in right side

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with operative bed seroma measures 14 x 50 mm with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with operative bed seroma with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Yours truly

Signature Dr. Wafaa Ali

01/09/2024 12:54 PM

Signature Dr. Wafaa Ali

01/09/2024 12:54 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

GOMAA, TAHANY Patient ID: 10110224008309 DOB: 21-Nov-1973 Date: 12-Sep-2024
Patient Lastname: Gomaa
Patient Firstname: Tahany
HIS Patient ID: 10110224008309
RIS Patient ID: 359100
Date of Birth: 21/11/1973

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

========== REPORT TEXT ==========
99mTc MDP Bone Scan Report (12/9/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

" No definite evidence of active osseous deposits seen all over the surveyed skeleton.
" Degenerative changes over the dorso-lumbar vertebrae.
" Arthritic changes are noted over both shoulder and both knee joints.
" Both kidneys are visualized.
Conclusion: -
" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged
Mahmoud Akram
Mohamed Abdeltawab, MSc

========== REPORT TEXT ENDE ==========

Bone Scan
[[DOCTYPE:BONE_SCAN]]

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ادارة الاشعة

Patient Name: Tahany Gomaa Kamel Sayed	Date : 28/08/2023
Exam : C.T CHEST	Age : 49 Year(s)
Medical ID: 3266/2023	

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Findings:

- Right breast soft tissue mass lesion measures about 4x3.5cm
- Clear both lung fields
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Upper abdominal cuts are unremarkable.

OPINION:

- ***Right breast soft tissue mass lesion***

Otherwise, normal CT study of the chest.

Dr.Shaimaa Megahed,MSc

Signature Prof. Amr Farouk
02/09/2023 09:40 ص
Dr. Amr Farouk, M.D.

Signature Prof. Amr Farouk
02/09/2023 09:42 ص
Dr. Amr Farouk, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

CAIRO UNIVERSITY HOSPITALS
FACULTY OF MEDICINE
RADIO DIAGNOSIS DEPARTMENT

مستشفيات جامعة القاهرة
كلية الطب
قسم الأشعة التشخيصية

WOMEN IMAGING DEPARTMENT

Patient Name: Tahany Gomaa Kamel

Date: **1-4-2024**

SECOND ATTEMPT WIRE LOCALIZATION USING A GRID

- *Two new wires were inserted to mark the two clips located at the right UOQ, both were directed from superior to inferior (perpendicular to both clips), both are about 2cm from the skin entry and marked with a blue circle on the skin around them and attached together by an adhesive tape.*
- *A trial for removing the two initially inserted wires was done (since they were very superficial) with successful removal of the posterior more superficial one and failed removal of the other, both skin entries were marked by a blue cross on the skin.*
- *A confirmatory mammographic image was taken showing the anterior wire exactly located at the clip site with the posterior one located within 9mm above the clip.*

NB: *An Initial attempt for wire insertion was done using 2 wires with stereotactic technique The 2 wires were inserted from superior to inferior targeting the clips. After we finishes they were seen in the ML view 2 cm , and 3.8 cm from the clips.*

Dr . Heba Azzam, MD
Dr. Iman Faker, MD
Dr. Lamiaa Bassam, MD
Dr.Yasmeen Essam Khalifa, MD
Dr. Safaa Ibrahim, MD

CT Scan
[[DOCTYPE:CT_SCAN]]

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ادارة الاشعة

Patient Name: Tahany Gomaa Kamel Sayed	Date : 04/03/2024
Exam : MAMMOGRAPHY BOTH	Age : 50 Year(s)
Medical ID: 3266/2023	Pat. ID: 41491

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

22 Rt CA
Sonno
2X27

Indication:

Right breast carcinoma on neoadjuvant

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B).
- Right breast UOQ 3 metallic clips on top of faint asymmetrical parenchymal density.
- No left suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed non appreciated right breast UOQ masses.
- Normal skin thickness and nipple-areola complex.
- Nonspecific axillary LNs.

Opinion:

- Status follow u post neoadjuvant chemotherapy showing right breast complete radiologic response with no currently appreciated sizable masses BIRADS-6

Yours truly

Signature Dr. Mohamed Mohsen

04/03/2024 3:17 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

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مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Tahany Gomaa Kamel Sayed	Date : 16/09/2024
Exam : MRI LUMBER SPINE	Age : 51 Year(s)
Medical ID: 3266/2023	Pat. ID: 41491

MRI OF THE LUMBOSACRAL SPINE

TECHNIQUE:

- Sagittal T1 and T2 WIs.
- Axial T1 and T2 WIs.

FINDINGS:

- Straightened lumbar curvature.
- Normal dimensions of the lumbar spinal canal.
- Mildly reduced bright T2 signal of the lumbar discs (except for L5-S1) denoting their degeneration.
- L4-5 posterior disc bulge with small central caudally migrated disc herniation seen effacing the anterior epidural fat, indenting the theca and partially encroaching on the corresponding neural foramina
- Normal size and signal of the conus medullaris.
- No marrow infiltrative lesions.
- No para-spinal soft tissue masses or collection.

OPINION:

- **L4-5 posterior disc bulge with supper added small central posterior caudally migrated disc herniation.**

Dr, Hala Sayed , MSc

Dr. Sherif Sameh, M.D.

M.R.I
[[DOCTYPE:MRI]]

Presidential Women's Health Outreach Program

Name: Tahany Gmaa Kamel

Date: 06/08/2023

Bilateral Full field Digital Mammography and High Resolution Ultrasound Examination

49 years old with right palpable lump

Detailed Findings

Right breast

- Heterogeneous fibroglandular pattern (ACR.c)
- Non circumscribed dense irregular mass seen at UOQ with internal microcalcifications
- **Ultrasound** demonstrated a corresponding irregular hypoechoic solid mass lesion with internal echogenic foci of calcifications seen at 10:00 outer zone c measuring about 3.2x3cm about 0.5cm from the skin. High internal vascular signal depicted by color Doppler study
- Another similar smaller mass seen 2cm lower to the aforementioned mass measuring about 1cm in diameters
- Normal skin and nipple areolar complex
- No pathological axillary lymph nodes.

Left breast

- Heterogeneous fibroglandular pattern (ACR.c)
- No masses, asymmetry, distortion or suspicious clusters of microcalcifications.
- **Ultrasound** demonstrated no solid or cystic mass lesions.
- Normal skin/nipple/areolar complex.
- No pathological axillary lymph nodes.

Opinion:

- **Right breast: (BIRADs 5)** UOQ highly suspicious multifocal solid mass lesions for biopsy and histopathological assessment.
- **Left breast: (BIRADs 1)** Normal study.

Best regards,
Dr. Eman Ali

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Tahany Gomaa Kamel Sayed	Date : 28/08/2023
Exam : ultrasound ABDOMEN& PELVIS	Age : 49 Year(s)
Medical ID: 3266/2023	Pat. ID: 41491

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size (15,5 cm at Rt MCL) bright parenchymal echopattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic billiary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- Average distention and capacity with no stones or mud
- *. The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

Average size , homogenous.

- **Kidneys: -**
- *Both are average in size,shape and echogenicity*
- *No stones or dilated pelvicalyceal system*
- *No gross lesion*

***Para-aortic and pancreatic area**: show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Average capacity with no stones or diverticular out pouching*

***Uterus and adenexeal** ; no abnormalities could be detected*

Opinion:

Bright liver likely fatty

Signature Dr . Nermien Ramadan
PM 2:37 2023/08/28

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:

Patient Name: *Tahany GOMAA KAMEL*	***Date of birth:*** *21/11/1973*
Arabic Name: تهانى جمعه كامل	***Patient Gender:*** *F*
Patient RIS ID: *359100* ***Patient HIS ID:*** *10110224008309*	***Exam Date:*** *27/07/2025*
Written by: Dr. Hoda Fathy	**Approved by:** Dr. Hoda Fathy

^{99m}Tc MDP Bone Scan Report (27/07/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP

Revealed:

Compared to bone scan dated (12-9-2024) at NCI, the current study reveals:-

- Still no definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

Follow up bone scan of managed breast cancer case shows: -

- Still no definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Bone Scan
[[DOCTYPE:BONE_SCAN]]

[[DOCTYPE:MAMMOGRAM]]
Mammogram

Medical No. : 11165
Patient's Name: Tahani Gomaa Kamel Sayed
Date: 2026-10-22
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Status post right CBS & ALAND, followed by radio therapy, routine follow up study.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- Thickened skin and coarsen trabeculae.
- Outer architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.
- Benign microcalcifications .
- **Ultrasound** demonstrated upper outer area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
- Thickened skin and coarsen trabeculae.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion: -

- ***Right breast (BIRADS 2): Post-Therapeutic sequalae.***
- ***Left breast (BIRADS 1): Normal study***

Recommendation: -

- ***For annual follow up.***

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%